

HEPATITIS B (HBV)

- Test for hepatitis B: send blood for hepatitis B surface antigen (HBsAg) if:
- Jaundiced (yellow skin/eyes)
 - ALT raised
 - HIV positive starting ART
 - As part of post/pre-exposure prophylaxis (PEP/PrEP) workup
 - Contact¹ of person known with hepatitis B
 - Pregnant (at booking visit)

- If patient has yellow skin or eyes, jaundice likely, assess and manage → 79.
- If not done already, also test for HIV → 110 and syphilis → 53. Manage further according to HBsAg result:

HBsAg positive

Patient has hepatitis B infection

- Notify.
- Educate that infection requires no specific treatment at this stage. Advise to return if jaundice develops.
- Educate that hepatitis B spreads via blood and sexual fluids. Advise patient to:
 - Reliably use condoms. Advise partners to test.
 - Avoid sharing toothbrushes, razors or needles. Advise household contact/s and needle-sharing/sexual contact/s to test.
- If HIV positive: ensure patient on ART containing tenofovir (TDF) and lamivudine (3TC) or emtricitabine (FTC). If not, discuss with experienced ART clinician or HIV hotline → 178.
- Also screen for hepatitis C: send blood for anti-HCV (EIA-antibody) or do rapid test, if available, using blood or saliva. If positive, refer.
- If pregnant and HIV negative: consider need for PrEP (do ALT) → 106. Tenofovir/emtricitabine (TDF/FTC) will also function as, treatment to prevent transmission of hepatitis B to baby. If needed, discuss with doctor/specialist.
- If able, arrange delivery at facility where hepatitis B immunoglobulin (HBIG) and hepatitis B monovalent vaccine available, → 167.
- Explain that hepatitis B infection can resolve by itself or become a chronic infection.

Check HBsAg after 6 months:

HBsAg positive

Patient has chronic hepatitis B infection

- Educate that chronic hepatitis B infection can lead to liver disease and cancer.
- Advise to avoid/reduce alcohol intake.
- Test for HIV:
 - If HIV positive:
 - Explain that certain medications used in ART will treat hepatitis as well. These will lower the hepatitis viral levels so that risk of liver disease is lowered.
 - Ensure patient on ART containing TDF + 3TC/FTC. If not, discuss with experienced ART clinician or HIV hotline → 178.
 - If HIV negative, refer for further tests and management of chronic hepatitis B infection.

HBsAg negative

Hepatitis B infection has resolved.

No further treatment needed.

If high risk lifestyle² advise to repeat HBsAg yearly.

HBsAg negative

Patient does not have hepatitis B.

- Any of:
- Health worker³
 - HIV positive
 - Contact¹ of person known with hepatitis B
 - Person who injects drugs (PWID)
 - Man who has sex with men (MSM)
 - Sex worker
 - Pregnant

No

Yes

- If not known to be immunised⁴, send blood for HBsAbs⁵:
 - If HBsAbs ≥ 10: patient is immune, no further management needed.
 - If HBsAbs < 10: give 3 doses of **hepatitis vaccine** 1mL IM at 0, 1 month and 6 months. Re-check HBsAbs 2 months after last vaccine:

HBsAbs ≥ 10

HBsAbs < 10

- Offer re-vaccination: give 3 doses of **hepatitis vaccine** 1mL IM, one month apart.
- Repeat HBsAbs two months after last vaccine given:

HBsAbs ≥ 10

HBsAbs < 10

Patient is immune due to previous hepatitis B vaccination. No further vaccination needed.

Repeat HBsAg test and discuss/refer.

Manage the baby born to mother with hepatitis B infection → 167.

¹Contact refers to household contact or needle-sharing/sexual partner of person known with hepatitis B (HBsAg-positive). ²New/multiple sexual partners, unprotected sex, exposure through skin like tattoo, piercing, sharing needles/other sharps.

³This includes student health care workers, clinic support staff (cleaners) and laboratory staff. ⁴Patient has no documentation of hepatitis B vaccination (e.g. Road to health booklet) or was born before April 1995 when hepatitis B vaccine was introduced into expanded programme on immunisation (EPI). ⁵HBsAbs - hepatitis B surface antibodies.